

INGUINAL HERNIA

Mr. Ramu 48/ M coming from Kolathur who is a Construction Worker came to OP with

Chief complaints

Swelling in the Left groin for past 2 months

History of presenting illness

Patient was apparently normal before 2 months.

When he noticed a swelling in the left groin.

Initially small

Gradually increased in size and attained the present size

Appears and Increases in size on lifting weights

Disappears on lying down

Associated with Pain over the swelling ,Intermittent,dragging type,For past 20 days

Precipitated on doing work,Relieved by rest,Not radiating

H/o Lifting weights present

No history of Trauma

No history of Chronic cough

No history of Constipation.

No history of increased frequency, urgency, or difficulty in micturition.

No history of abdominal distension.

No history of vomiting, colicky abdominal pain

Past History

No history of similar complaints in the past.

Not a known Diabetic, Hypertensive, Tuberculosis, Asthma, Epilepsy.

No history of previous surgery or hospitalisation

No history of blood transfusion

Personal history

Patient is on mixed diet.

Consumes alcohol for past 10 years. Brandy 180ml/ day

Patient has quit smoking 15 years back, before which he smoked 4 bidis / day

Bowel and bladder habits normal.

Family History - No significant family history

General Examination

Patient was conscious, oriented, co-operative, comfortable at rest

Afebrile

Moderately built and moderately nourished

No pallor

Not icteric

No cyanosis, clubbing

No pedal edema

No Significant Generalized lymphadenopathy

Vital Signs

Pulse rate : 81/min regular, normal volume, no specific character, felt in all peripheral pulses

Respiratory rate : 17/min

Blood pressure : 130/90 mm Hg

Temperature : 37.4 degree Celsius

Examination of inguino scrotal region

Inspection

After consent, Pt exposed from umbilicus to Mid thigh, Pt examined in Standing position.

Single swelling is seen in the Left inguinal region
Approximately 5x5 in size
Hemispherical in shape
Situated about 7 cm from ASIS and Just above the root of the scrotum.
Surface is smooth
Well defined margins
Skin over the swelling is normal
Cough impulse is present
No visible Peristalsis, No scars, sinuses or dilated veins.
Penis in midline
Urethral meatus normal
Scrotum appears normal.
Tone of the abdominal muscles are normal.

Palpation

Not warmth, Not tender
A single swelling of size 5x5 cm in the Left Groin
Hemispherical in shape
Extending about 7 cm from the ASIS and lower border is just above the root of the scrotum
Surface is smooth
Skin over the swelling is normal
Soft and Elastic in consistency
Propulsive Expansile cough impulse felt
Swelling is reducible
Urethral meatus normal; no strictures
Both testis palpable

DEEP RING OCCLUSION TEST – swelling does not appear

ZIEMAN TECHNIQUE - impulse felt at the Index Finger

FINGER INVAGINATION TEST - Impulse felt at the Tip of the Little finger

Per rectal examination: To be done

Percussion

Resonant note heard over the swelling

Ascultation

Normal bowel sounds heard

Other system Examination

Abdomen : Soft, non-tender, no free fluid and no organomegaly

RS : NVBS heard and no added sounds.

CVS : S1 S2 heard , no murmur

normal bowel sounds heard.

CNS: No focal neurological deficit

Diagnosis

Left sided Incomplete Indirect Inguinal hernia with bowel as its content without any complications.

Investigations

Specific – USG Abdomen and pelvis

Routine

Complete blood count

Random blood sugar, fasting and postprandial blood Sugar

Blood urea & Serum Creatinine

Urine – Sugar, albumin and Deposits

Chest X-ray PA view

ECG

HIV ELISA

HbsAg Surface Antigen

Treatment

Left sided lichenstein tension free open mesh repair hernioplasty

CA BREAST

Mrs.Lakshmi 54/F lower seller residing at Thirumullaivoil came to OP with

Chief complaints

Lump in the Rt breast for past 1.5 months

History of presenting illness

She was apparently normal before 1.5 months.

Afterwhich she developed a lump in the Rt breast which is insidious in onset,initially small in size and progressed to the present size.

The lump is associated with dull aching pain for past 1.5 months ,intermittent in nature ,No radiation.The pain is severe for past 3 weeks.

No H/o Nipple discharge

No H/o recent retraction of nipple.

No H/o trauma

No H/o Swelling elsewhere in the body

No H/o fever

No H/o loss of weight,loss of appetite

No H/o bony pain

No H/o cough,hemoptysis,breathlessness

No H/o Jaundice

Past History

No H/o Previous similar illness

She is a k/c/o Diabetes mellitus for the past 3 year taking oral hypoglycemic medications and under control

H/o sterilisation surgey at 19 years of age

Not a k/c/o Hypertension, Epilepsy,Bronchial asthma,IHD.

Personal History

Non smoker, Non alcoholic

Takes non-veg diet

Normal sleep pattern

Normal bladder and bowel movements

Menstrual History

Attained menarche at 14 yrs of age.

Regular cycles 5/30, Normal flow, No clots

Attained menopause at 50 years of age.

Marital and Obstetric history

Married at 15 years of age. Non-consanguineous marriage

First child birth at 16 years of age - Normal vaginal delivery- breast fed for 3 years

Second child birth at 19 years of age - Normal vaginal delivery- breast fed for 1 year.

No relevant Family history

EXAMINATION

General Examination

Conscious and oriented

Moderately built and nourished.

No pallor, icterus, cyanosis, clubbing, jaundice, General lymphadenopathy, Pedal edema.

Vital signs

Pulse rate - 68 beats/min

BP - 110/74 mm Hg

Resp rate - 16 breaths/min

She is afebrile

Local examination

After consent and with female attender the patient is asked to expose up to waist.

Inspection - On sitting posture arms by the side of the body

Position, symmetry, and Size of the Rt is normal compared to the opp breast

No obvious lump made out on inspection.

No ulcer present

Skin over the breast normal

No dilated veins

No dimple/puckering/retraction

No peau d'orange appearance

No nodules, ulceration, fungation.

Rt nipple lies at a higher level compared to normal side.

Areola is normal.

Arms raised above the head

No change in the shape of the breast after lifting upwards.

No tethering, fixing

Inframammary region was normal, Axilla normal.

Leaning forward

The lump is not fixed to the chest wall.

Falls equally on both sides.

Palpation

No warmth, Tenderness present.

Solitary lump of size 7 x 8 cm is present over the inner upper and inner lower quadrants of the Rt breast with uneven shape, ill-defined margins, hard in consistency

Not fixed to the skin

Mobile with the breast tissue

Fixed to the breast tissue

Not fixed to pectoralis muscle

Not fixed to the chest wall

Rt Nipple is harder compared to normal side.

Lymph note

Central, axillary lymph node +

Single, 2x2, Firm in consistency and mobile.

Examination of the opposite breast, axilla, and inframammary region normal.

Other system

Abdomen - Soft, No tenderness, No organomegaly, No clinically palpable mass.

RS - BAE + , Normal vesicular breath sounds

Musculoskeletal system - Normal

CVS - S1, S2 heard , No murmurs

Summary

Mrs.Lakshmi 54 y/o post menopausal women with % painful Lump in Rt breast and on examination a solitary hard lump which is fixed to the breast tissue with axillary lymph node.

Diagnosis

Mrs.Lakshmi 54 y/o probably a case of carcinoma Rt breast -T3 N1 Mx corresponds to stage IIIA

Investigation

Specific investigation

Mammogram

True cut biopsy

USG breast

MRI breast

Staging investigation

USG abdomen

Bone scan

CT chest

Routine investigation

Complete blood count

Blood urea,Creatinine

LFT

Fasting and PP blood glucose

Urine albumin sugar

Blood grouping and typing

Serology - HIV,HbsAg,HCV

X ray chest,ECG all leads

Echo

Treatment

Modified Radical mastectomy and sample to frozen section with chemotherapy and hormonal therapy follow-up.

THYROID

Mrs.Chandra 50/Fwho is a House wife comming from Korukkupet came with
Chief complaints of

Swelling in the front of the neck for past 4yrs

History of presenting illness

The patient was apparently normal before 4yrs after that she noticed small swelling on the anterior aspect of the neck which increased in size for past 5months and reached the current size

Which is insidious in onset, progressive in nature, not associated with pain.

No h/o any other swelling

No h/o fever

No History suggesstive of pressure symptoms like

No h/o dyspnoea

No h/o dysphagia

No h/o change in voice

No h/o suggesstive of horner syndrome

No h/o syncope

No History suggesstive of hyperthyroidism like

No h/o excess sweating

No h/o loss of weight inspite of good appetite

No h/o heat intolerance

No h/o diarrhoea

No h/o sleepless nights

No h/o muscle weakness

No h/o tremors

No h/o palpitations

No h/o dyspnoea on excertion

No History suggestive of hypothyroidism like

No h/o weight gain inspite of poor appetite

No h/o constipation

No h/o cold intolerance

No h/o easy fatiguability

No history suggestive of malignancy

NO h/o sudden increase in size

No h/o loss of appetite and weight loss

No h/o bone pain

No h/o jaundice

No h/o cough with hemoptysis

No h/o headache, convulsions, seizures

Past History

No h/o similar complaints in the past

Not a known case of diabetes , hypertension, asthma , epilepsy, tuberculosis, jaundice , ischaemic heart disease

No h/o irradiation

No h/o previous surgery

No h/o drug intake

Personal History

Mixed diet

No adverse social habits

Takes Iodised salt

No h/o excessive intake of goitrogens

No h/o altered sleep pattern

Menstrual History

Attained menarche at 15yrs of age

Normal flow, regular

3/30days cycle, not associated with pain and clots

Attained menopause 10yrs before at the age of 40

Family History

No history of similar complaints in the family

General examination

Conscious, oriented, comfortable, co-operative,

Moderately built and nourished

afebrile

No pallor

Not icteric

No cyanosis

No clubbing

No pedal edema

No peripheral significant lymphadenopathy

Eyes-normal

VITALS

BP: 130/80mm Hg measured in left upper limb in sitting posture.

Respiratory rate: 17/min thoracoabdominal

Pulse rate: 71/min regular in rhythm, normal in volume, no specific character, no radio-radial and no radiofemoral delay, felt in all accessible peripheral vessels.

Temperature: 98⁰ F

INSPECTION

Swelling in the anterior aspect of the neck in the region of thyroid of size 6*4cm

Lower border is seen

Swelling moves with deglutition

Oval in shape

Surface appears to be smooth

Margins well defined

Skin over the swelling normal

No scar , no sinus, no dilated veins

No visible pulsations

Trachea appears to be in midline

PALPATION

Not warm , not tender

Swelling of size 6*4cm in the anterior aspect of neck on the region of the thyroid, oval in shape,

Lower border extending 2cm away from suprasternal notch , upper border extending 1cm from hyoid bone , right side – lies close to anterior border of sternocleidomastoid, left side – 2cm away from the anterior border of sternocleidomastoid

Surface nodular, Firm in consistency

Plane of the swelling : deep to deep fascia

Carotid pulsation felt against the upper border of thyroid cartilage equal intensity on both sides

No palpable thrill

Trachea is in midline

No other swellings present

PERCUSSION

No retrosternal dullness

AUSCULTATION

No bruit heard

EXAMINATION OF LYMPH NODES

No palpable lymph nodes in the neck region.

OTHER SYSTEM EXAMINATION:

RS: Normal vesicular breath sounds heard

CVS: S1 , S2 heard, no murmur

CNS : No focal neurological deficit

Spine and cranium normal.

SUMMARY

50 year old female with complaints of swelling in the anterior aspect of the neck for the past 4 years with no history of pressure symptoms, hypothyroidism, hyperthyroidism, malignancy and metastasis.

ON EXAMINATION, 6*4 cm , firm, nodular swelling is present in the anterior aspect of the neck in the region of thyroid.

Diagnosis

A Case of nontoxic nonmalignant multinodular goiter without complications

INVESTIGATION

Specific investigations

Thyroid profile

USG neck

Fine needle aspiration cytology.

Indirect laryngoscopy/ video laryngoscopy & Routine investigations

TREATMENT

Total thyroidectomy

OBSTRUCTIVE JAUNDICE

Mr . Azhagappan 57/M from ariyaloor who is a farmer came with

Chief complaints of

- Pain in the abdomen for the past 3 weeks
- Yellowish discolouration of the eyes for the past 2 weeks

History of presenting illness

Patient was apparently normal before 3 weeks ,after which he developed

- Pain in the abdomen for past 3 weeks
 - acute onset
 - dull aching type
 - intermittent pain
 - more in the right upper abdomen
 - radiating to the back
 - not related to food intake
- Yellowish discolouration of the eyes for past 2 weeks
 - Insidious onset,
 - progressive in nature
 - No waxing or waning
- H/o passing high coloured urine for the past 2 weeks
- H/o passing clay coloured stools for the past 2 weeks
- H/o Itching all over body for the past 2 weeks started in the leg
- No h/o fever.
- No h/o abdominal distension.
- No h/o nausea/vomiting.
- No h/o belching
- No h/o loss of weight

- No h/o loss of appetite
- No h/o hematemesis
- No h/o maelena or hematochezia.
- No h/o other bleeding tendencies.
- No h/o steatorrhoea
- No h/o altered bowel habits
- No h/o worms in stools.
- No h/o dyspnoea, cough, hemoptysis
- No h/o headache, blurring of vision, convulsions
- No h/o back pain

Past History

- No h/o similar complaints in the past.
- No h/o Diabetes mellitus , Hypertension, cardiac diseases, asthma, tuberculosis
- No h/o previous abdominal surgeries
- No h/o any chronic drug intake.

Personal History

- Mixed diet
- Consumes alcohol since 35 years of age till the onset of symptoms 180ml (5 units)in 2-3 days a week.
- Smokes since 35 years of age till the onset of symptoms , 10-20 beedi per day

smoking index : 440

pack years : 20

- Normal sleep and appetite
- Normal bowel and bladder habits

Family History

- No similar complaints among family members.
- Born out of non consanguinous marriage
- Married and has 2 children
- No cancer related deaths in the family
- No metabolic diseases in the family

Treatment History

- Undergone native drug treatment for jaundice

3 days after the onset of symptoms, discontinued after 3 days due to lack of improvement

Summary

- A 57 year old male Mr Azhagappan, belonging to lower middle class socioeconomic status, has come with chief complaints of right upper abdominal pain , jaundice, high colored urine , pale stools and itching . So I would like to examine abdominal system.

General Examination

- Examination done after getting consent from the patient, in a well lighted and ventilated room, after ensuring the privacy of the patient, stripping the patient upto mid thigh.

Patient is conscious, oriented, comfortable, moderately built and ill nourished

Hydration good

Icteric present.

No pallor.

No cyanosis.

No clubbing.

No generalised significant lymphadenopathy.

No pedal edema.

Vital Signs

BLOOD PRESSURE :130/80mm Hg, in left upper limb in sitting posture.

PULSE RATE:90/min, regular in rhythm, normal volume, no special character, no radio radial delay, no radio femoral delay , no vessel wall thickening.

RESPIRATORY RATE: 18/min , abdomino-thoracic type.

TEMPERATURE: Afebrile

- Eyes icteric
- Undersurface of tongue yellowish discolouraton
- Scratch marks seen on abdomen and lower limbs.
- No other signs of liver cell failure
- Alopecia
- Madarosis
- Bitot's spot
- Loss of buccal pad of fat
- Bilateral Parotid enlargement
- Gynaecomastia
- Spider navi
- Dupuytren's contracture
- Palmar erythema
- ascitis
- Testicular atrophy
- Pedal edema

Examination of Abdomen

Inspection

- Abdomen contour normal, no localised or generalised distension, flanks free
- Umbilicus in midline.
- All quadrants moves with respiration
- No visible peristalsis and pulsations.
- Skin over the abdomen - Scratch marks present, no scars, no sinuses, no dilated veins
- Hernial orifices are free.
- External genitalia appears to be normal.

- Renal angle – no fullness.
- Supraclavicular area- no fullness

Palpation

Position – supine with knees flexed, arms by the side of the body

- No warmth
- **Tenderness over the right hypochondrium region**
- No guarding/ rigidity
- **A vague mass in the right hypochondrium region**

size of mass – 8x5cm, ill defined margins, surface is smooth, firm in consistency, moves with respiration, less prominent on head raising.

- No other mass
- No organomegaly
- External genitalia – normal.
- Hernial orifices are free.
- Renal angles are free

Percussion

Liver span normal

Dullness over the mass

Tympanic over the other areas of abdomen

No free fluid

Ascultation

Normal bowel sounds heard.

No bruit

No venous hum

Examination of Lymph nodes

No palpable PARAUMBILICAL & LEFT SUPRACLAVICULAR lymph nodes

Per rectal Examination – to be done

OTHER SYSTEM EXAMINATION

RS: Normal vesicular breath sounds heard

CVS: S1 , S2 heard, no murmur

CNS : No focal neurological deficit

Spine and cranium normal.

Provisional Diagnosis

A case of obstructive jaundice for evaluation probably due to distal common bile duct obstruction may be due to malignancy without any inflammation.

DIFFERENTIAL DIAGNOSIS

- Periapillary Carcinoma
- Carcinoma head of pancreas
- Cholangiocarcinoma
- Double impaction of stone

GENERAL INVESTIGATIONS

Complete hemogram : Hb%, TC, DC, ESR

Random blood sugar

Renal function test, Electrocardiogram

Chest X ray

HIV, HbsAg

SPECIFIC INVESTIGATIONS

- LFT
Total bilirubin, Direct bilirubin, SGOT, SGPT, ALP, GGT, S. albumin
- Urine – bile salts, bile pigments, urobilinogen
- Coagulation profile – BT, CT, PT, aPTT, INR

- CA 19-9 ,CEA
- Serum amylase ,lipase

Imaging studies

- USG abdomen
- CECT
- ERCP , MRCP
- MRI

SURGERY

A. OPERABLE: whipple's procedure

INOPERABLE: palliative roux en y choledochojejunostomy along with gastrojejunostomy & cholecystectomy

ERCP & stenting

B. cholecystectomy with bile duct exploration followed by kehrs T-tube insertion for drainage.

PERIPHERAL VASCULAR DISEASE

A 30 years old male Mr. Prakash coming from Mint st. working in building construction company Belonging to low socio economic status came with

CHIEF COMPLAINTS of

- Pain in the left leg for past 5 months.
- Ulceration of the left little toe for the past 4 months.
- Blackish discoloration of little toe for the past 4 months.

HISTORY OF PRESENTING ILLNESS

Patient was apparently normal before 5 months. Then he developed

- H/o Pain in the left leg for the past 5 months.
 - The patient after walking for a distance of about 50 m develops pain for which he takes rest for about 2 minutes and then resumes walking.
 - Type of pain- dull aching pain
 - Not radiating
 - Aggravated by walking and cold exposure
 - Relieved by rest.
- H/o Rest pain.
- H/o Sleep disturbance due to pain
- H/o Blackish discolouration of left little toe for past 4 months.
- Started as a small blister, ruptured to form an ulcer in 10 days then developed a blackish discolouration.
- No H/o trauma
- No H/o fever
- No H/o superficial thrombophlebitis
- No H/o cardiac problems
- No H/o chest pain
- No H/o fainting

- No H/o black outs
- No H/o abdominal pain
- No H/o impotence
- No H/o blurred vision.

PAST HISTORY:

- No H/o similar episodes in the past
- No H/o diabetes mellitus, hypertension, tuberculosis, asthma, epilepsy, cardiac problems
- No H/o jaundice
- No H/o blood transfusion
- No H/o previous hospitalisation
- No H/o previous surgeries

FAMILY HISTORY:

- No H/o similar complaints in the family members
- No H/o atherosclerotic diseases in the family
- No H/o metabolic disorders and malignancies running in the family members

PERSONAL HISTORY:

- Born out of Non Consanguineous Marriage.
- He takes mixed diet
- H/o smoking for the past 6 years - 10 cigarettes per day.
smoking index – 60
- H/o drinking alcohol, drank brandy 180 ml/day for 5 years
- H/o tobacco chewing for the past 6 months, 2 to 3 times per week.
- No H/o extra-marital affairs

- **TREATMENT HISTORY:**

- No H/o any drug intake

- **ALLERGY HISTORY:**

- No H/o any drug and food allergies

GENERAL EXAMINATION:

- Conscious and oriented
- Moderately built and moderately nourished
- No pallor
- No icterus
- No cyanosis
- No clubbing
- No significant generalised lymphadenopathy
- No pedal edema

VITALS:

- Pulse rate: 92/min
- Respiratory rate: 14/min
- Blood pressure: 110/80 mm Hg measured in sitting position on right upper limb
- Temperature: Afebrile.

LOCAL EXAMINATION:

INSPECTION: of Left lower limb in supine position

- Attitude – Normal
- No deformity of toes
- No muscle wasting
- No guttering of veins
- Skin-

- thinning of skin – present
- loss of hair
- loss of shininess
- loss of subcutaneous fat.
- Nail- brittle and transverse ridges seen.
- A scar of size 4x4 cm present on the dorsum of foot 2 cm lateral to the medial malleolus.
- Dry gangrene over the fifth and third toe of left foot on the plantar aspect with well defined line of demarcation.
- Superficial thrombophlebitis present

PALPATION

Skin- cold

Gangrenous area

- Site- Ball of left fifth toe and third toe
- Dry and hard
- Sensation- present,
- Surrounding area is hyperaesthetic
- No local crepitus

Movements of joints adjacent to gangrenous area- Normal.

EXAMINATION OF OTHER LIMB – NORMAL

Examination of Pulse

ARTERIES	RIGHT SIDE	LEFT SIDE
CAROTID ARTERY	+	+
SUBCLAVIAN ARTERY	+	+
SUPERFICIAL TEMPORAL ARTERY	+	+
AXILLARY ARTERY	+	+
BRACHIAL ARTERY	+	+
RADIAL ARTERY	+	+
ULNAR ARTERY	+	+
FEMORAL ARTERY	+	+
POPLITEAL ARTERY	+	+
POSTERIOR TIBIAL ARTERY	+	–
ANTERIOR TIBIAL ARTERY	+	+
DORSALIS PEDIS ARTERY	+	+

AUSCULTATION:

No bruit heard.

NEURO EXAMINATION

- MOTOR SYSTEM– Normal
- SENSORY SYSTEM- Normal

EXAMINATION OF LYMPH NODES:

No clinically palpable lymph nodes.

OTHER SYSTEM EXAMINATION:

CVS: S₁ and S₂ heard, no murmurs

ABDOMEN: Soft, no organomegaly

RS : NVBS heard

CNS : No focal neurological deficit

Spine and cranium : Normal

DIAGNOSIS:

Critical Limb ischemia causing Gangrene of Left fifth and third toe due to ThromboAngitis Obliterans-Grade III, category 5 block at the level of Posterior tibial artery with known risk factor of Smoking and tobacco chewing.

INVESTIGATIONS:

I) GENERAL

Blood : Complete blood count, ESR,

sugar, lipid profile,

coagulation profile,

urea

Plasma fibrinogen

Serum : protein S and C, creatinine

Chest X ray

ECG

II) SPECIFIC:

- Doppler scan (Ankle Brachial Index)
- Duplex scan
- Ultrasound Abdomen
- Angiography
 - Retrograde transfemoral seldinger angiography
 - CT Angiography
 - MRI Angiography
 - Digital subtraction angiography

TREATMENT:

1) General measures:

- Abstinence from smoking
- Life style modification
- Regular controlled walk and diet
- Buerger's position and exercise
- Care of the foot

2) Medical:

- Pentoxifylline- 400mg TDS for 3-4 weeks
- Cilostazole – 100mg bid
- Analgesics
- Low dose Aspirin – 75 mg od
- Clopidogrel – 75mg od
- Antibiotics
- Vasodilators

3) Surgical:

- Amputation of gangrene
- Lumbar sympathectomy
- Bypass graft
- Percutaneous transluminal balloon angioplasty with stenting.

RIF MASS

Mr.Kuppan,60 year old male,Farmer, coming from Ponneri,
came to the OP with **chief complaints** of

- ▶ Pain in the right lower abdomen for past 1and half months
- ▶ Lump in the right lower abdomen for past 1 month

History of Preseting Illness

Patient was apparently normal before 1 and half months

H/O Pain in the right lower abdomen

for past 1 and half months,
insidious in onset,
dull aching pain,
on and off,
no aggravating or relieving factors,
not radiating

H/O Lump in the right side of abdomen

for past 1 month
insidious in onset,
gradually progressed to the present size,
associated with pain

H/O Fever for past 1 month,

low grade,
intermittent,
evening rise of temperature,
not associated with chills and rigor

H/O Cough with expectoration for past 1 month

H/O Loss of weight

No H/O Loss of appetite

No H/O Vomiting

No H/O Abdominal distension

No H/O Constipation, Diarrhoea

No H/O Bleeding per rectum, blood in stools

No H/O Painful, increased frequency of urination, hematuria

No H/O Jaundice, bone pain

Past History

H/O Tuberculosis- 35 years back, treated with anti-tubercular drugs for 6 months

H/O Cataract surgery in the left eye 1 year back

No H/O Diabetes mellitus, Hypertension, Bronchial asthma, Epilepsy

No H/O Blood transfusion

Personal History

Married, living with wife and children, born out of non consanguinous marriage

Mixed diet

H/O Smoking for past 30 years, 4 beedis/day Smoking Index – 120

Patient was used to consume alcohol for 20 years, 180ml of brandy per day,
Stopped before 10 years

Normal bowel and bladder habits

ALLERGIC HISTORY No H/O Allergy to drugs or food.

FAMILY HISTORY: No significant family history

General Examination

Patient is conscious, oriented, afebrile

Ill built and ill nourished

Pallor present

No icterus

No cyanosis

Clubbing present- Grade 2

No pedal edema

No significant generalized lymphadenopathy

Vital Signs

Pulse rate: 86/min, regular in rhythm, normal in volume, no specific character, no radio-radial or radio-femoral delay, felt in all palpable peripheral vessels

Respiratory rate: 18/min

Blood pressure: 110/70 mm Hg, in right upper arm in sitting posture

INSPECTION:

Abdomen contour- Normal, umbilicus in midline and inverted

All quadrants move equally with respiration

No scar, sinus, dilated veins, visible pulsation

No fullness, visible gastric/intestinal peristalsis

Flanks free, hernial orifices free, external genitalia normal

Renal angle free

Left supraclavicular fossa- normal

Palpation

Patient in supine position with hips flexed

Not warm, not tender

A mass is felt in the *right iliac and lumbar region* of size 5×4 cm, **oval** in shape

Extends-medially 7cm from **umbilicus**

Laterally 6cm from **anterior superior iliac spine**

Superiorly 4cm from **right costal margin at midclavicular line**

Inferiorly 5cm from **pubic symphysis**

Firm in consistency, with smooth surface, with well defined margins

Freely mobile in all directions

Leg rising test (Carnett's test)- mass does not become prominent- Intra abdominal mass

Knee elbow test- mass falls forward

No guarding, rigidity

No other mass palpable

No organomegaly

Mass is not ballotable

Renal angle-normal

Left supraclavicular fossa-normal

Per rectal examination- not done

PERCUSSION:

Impaired resonance over the mass

Liver span – normal

AUSCULTATION:

Normal bowel sounds heard

Other System Examination

CVS- S1, S2 heard, no murmur

RS- Normal vesicular breath sounds heard

CNS- No focal neurological deficit, Higher functions- normal

Provisional Diagnosis

A case of Right iliac fossa mass for evaluation

DD

Ileocecal tuberculosis

Carcinoma caecum

Investigation

Specific Investigation

Ultrasound abdomen

Xray abdomen

CT scan abdomen

Mantoux, Sputum for acid fast bacilli, BACTEC

Barium meal follow through

Tumour markers- CEA, Alpha fetoprotein

Colonoscopy

Routine

Blood inv- Hemoglobin, Total count, Differential count, ESR, Blood grouping and typing

Blood sugar, Urea, Serum creatinine, Electrolytes

Urine- albumin, sugar, deposits

Stool- occult blood

Xray Chest, ECG

Treatment

ILEOCECAL TUBERCULOSIS:

Anti Tubercular drugs

Ileocecal resection

CARCINOMA CAECUM:

Right hemicolectomy

VARICOSE VEINS

Mr.Raghavan 65/M from Vyasarpadi who is a Tea master came to OP with **Chief complaints** of

DILATED AND TORTUOUS VEINS in the LEFT leg for the past 6 months.

BLACKISH DISCOLORATION in the lower third of left leg for the past 3 months.

ULCER in the left ankle for the past 1 week.

History of Presenting illness

- H/O dilated and tortuous veins in the INNER aspect left leg for the past 6 months
 - Insidious onset and progressive in nature.
 - Becomes more prominent on prolonged standing and less prominent on lying down.
 - Not associated with pain/night cramps
- H/O BLACKISH DISCOLORATION and ITCHING in the left leg for the past 3 months.
- H/O SWELLING in the left leg for the past 3 months
 - occurs during end of the day and relieved after a period of rest
 - Not associated with pain
- H/O ulcer over the INNER aspect of left ankle for the past 1 week
 - Following trauma by scratching which is due to presence of itching
 - Associated with scanty serous discharge
 - Not blood stained and not foul smelling
 - Not associated with pain
- No H/O fever
- No H/O prolonged immobilisation
- No H/O abdominal distension

- No H/O constipation
- No H/O trauma
- No H/O pain during walking

Past History

- No history of similar complaints in the past
- No history of any surgery in the past
- Not a known case of diabetes, hypertension, asthma, epilepsy, TB, ischemic heart disease.
- No H/O any prolonged drug intake.

Personal History

- He is on Mixed diet
- Alcoholic for the past 20 years
- Not a smoker
- No H/O any allergy
- Bowel and bladder habits normal

Family History - No similar complaints in the family.

Summary

- A 65 yrs old male working as a tea master came with complaints of **dilated and tortuous veins** in the inner aspect of left lower limb for past 6 months, **blackish discoloration** , swelling and itching for the past 3 months, **ulcer** for past 1 week with no other positive history.
- Conscious, Oriented and Comfortable
- Moderately built and Moderately nourished
- No pallor
- Not icteric
- No cyanosis
- No clubbing

- No significant generalised lymphadenopathy
- Pedal edema in left leg

Vital Signs

PULSE RATE:82/min, regular in rhythm, normal volume, no special character, felt in all peripheral vessels.

BLOOD PRESSURE:120/80mm Hg, measured in right upper limb in sitting posture.

RESPIRATORY RATE: 18/min , abdomino-thoracic type.

TEMPERATURE: Afebrile

INSPECTION : (in standing position)

- Dilated and tortuous veins seen over the ANTERIOR and MEDIAL aspect of knee and MEDIAL aspect of left leg
- HYPERPIGMENTATION over LOWER THIRD of left leg
- Edema is present below the left knee
- MULTIPLE ULCERS is present over the medial aspect of ankle
 - variable sizes(2 X 1 cm) , irregular in shape
 - Well defined margin
 - Sloping edge
 - Healthy granulation tissue is present on floor
 - No discharge
 - Surrounding skin shows hyperpigmentation
- No redness
- No scars seen
- No loss of hair and brittleness of nails
- No deformity present

PALPATION :

- No warmth

- No tenderness
- Lipodermatosclerosis present
- ULCER
 - 2 X 1 cm , irregular in shape
 - Well defined margin
 - Sloping edge
 - Healthy granulation tissue is present on floor
 - No discharge
 - base – not indurated , not fixed
 - does not bleed on touch
- PULSATIONS are felt in posterior tibial and dorsalis pedis artery
- Lymph nodes are NOT PALPABLE.
- **BRODIE TRENDELENBURG TEST 1** : positive
Fast filling indicates SFJ incompetence
- **BRODIE TRENDELENBURG TEST 2** : positive
Gradual filling of veins is seen below the knee.
- **MORRISEY'S COUGH IMPULSE**: positive Thrill present on coughing
- **MULTIPLE TOURNIQUET TEST** :
veins are seen at the region of below knee perforator
- **FEGANS TEST**
Depressions are felt in deep fascia below knee
- **SCHWARTZ TEST** : Impulse felt on saphenofemoral junction when percussed below.

Examination of other limb : NORMAL

OTHER SYSTEM EXAMINATION

Abdomen:

INSPECTION – No dilated veins, visible swelling or mass.

PALPATION – Soft, non-tender, no fluid thrill, no palpable mass

CVS- S1, S2 heard, no murmur

RS- Normal vesicular breath sounds heard

CNS- No focal neurological deficit, Higher functions- normal

Diagnosis

A case of primary varicose vein in **left** lower limb involving **Great saphenous vein** with below knee **Perforator incompetence** with **Chronic venous insufficiency**

CEAP classification : **C₆E_PA_{sp}P_R**

Investigations - Specific

- Duplex scan
- USG abdomen
- X ray ankle joint
- Routine investigations

Treatment

- **TRENDELENBERG SURGERY** – flush ligation with stripping of great saphenous vein
- Subfascial perforator ligation

GASTRIC OUTLET OBSTRUCTION

Mr. Selvam, a 46yrs old male patient coming from Tirutani who studied upto second standard and is a farmer by occupation.

CHIEF COMPLAINTS

H/O Vomiting for past 3 months.

History of presenting illness:-

- The patient was apparently normal before 3 months, after which he developed
- Vomiting for 3 months 5-6 episodes/day.
- Non-projectile vomiting.
- Contains partially digested food particles.
- Initially vomiting was after 1-2hrs of taking food and progressed to vomiting soon after taking food at present.
- Vomitus is not blood stained or bile stained.
- H/O sensation of fullness after taking food.
- H/O loss of weight.
- H/O loss of appetite.
- H/O ball rolling movements present.
- No H/O abdominal pain.
- No H/O hematemesis or melena.
- No H/O yellowish discoloration of eyes and urine.
- No H/O cough or hemoptysis.
- No H/O bone pain.
- No H/O headache, altered sensorium or seizures.
- No H/O fever.
- No H/O skin pigmentations
- No H/O altered bowel habits.

Past History

- No H/O similar episodes in the past.
- Patient is not a known case of HT/DM/TB/BA/IHD/HIV infection, jaundice or epilepsy.

Personal History.

- Consumes mixed diet.
- H/O irregular food intake.
- Pt is a chronic alcoholic. Consumes 90mL Brandy X 15yrs.
- Chronic smoker; 10 cigarettes/day X 15yrs. 15 packs years.

Family History

- No similar complaints among family members.

Treatment History:

- No H/O hospital admissions/prolonged treatment history.

Allergic History:

- No H/O any allergy to any foods or drugs.

General Survey:

- middle aged, patient dull looking , greying of hair present, eyes normal muddy conjunctiva , ear normal, oral cavity :dental hygiene poor nicotine stains present, gait normal

General Examination.

- Patient is conscious, cooperative, comfortable, oriented and afebrile.
- Poorly built and nourished.
- Pallor present.
- No icterus, no cyanosis, no clubbing, no generalised superficial lymphadenopathy, no pedal edema.

Vitals:

- B.P = 110/70 mm Hg measured in Right arm, sitting position.
- Pulse = 78/min Regular rhythm and normal volume.

No special character. No radioradial or radiofemoral delay.

Felt in all peripheral vessels.

- Respiratory Rate = 18/min, abdominothoracic type.
- Temperature = afebrile.

Local Examination: (Abdomen)**On Inspection:**

- Abdomen scaphoid in shape.
- Fullness is seen in the epigastric region.
- All quadrants move equally with respiration .
- Umbilicus in midline-inverted. No nodules or discoloration seen around umbilicus.
- No scars, no sinuses.
- No distended veins and no visible pulsations over anterior abdominal wall.
- Visible gastric peristalsis.
- No visible intestinal peristalsis.
- Hernial orifice free. External genitalia – Normal

On Palpation:

- On Superficial palpation:

- No local rise of temperature over abdomen.
- No tenderness.
- Abdomen is soft. No guarding or rigidity.
- Mass is felt in epigastric region.

- On Deep palpation:

- No tenderness.
- Hard irregular mass is felt in epigastric region extending to Right hypochondrium, below the costal margin of size 8 x 5cm.
- Upper border is not palpable.
- Mass moves with respiration.
- Mobility is restricted in vertical side to side.
- No fixity to skin.
- It becomes less prominent on head raising test.
- Liver and spleen not palpable.
- No palpable lymph nodes

On Percussion:

- Impaired resonant note over the lump.
- Succussion splash heard.
- No shifting dullness or fluid thrill- no free fluid.

On Auscultation:

- Auscultoscraping: Greater curvature of stomach is delineated. Stomach appears to be dilated.

Per rectal Examination: To be done

Other system Examination

- RS: Normal vesicular breath sounds heard.
- CVS: S1, S2 heard. No added sounds.
- CNS: No focal neurological deficit.
- Spine and Cranium Normal

Provisional Diagnosis:

- Gastric Outlet Obstruction with features of Carcinoma of Stomach.

Differential Diagnosis:

- Cicatrising duodenal ulcer.
- Ulcer at antrum of stomach.
- CA head of pancreas.
- Annular pancreas.
- Trichobezoar.

Investigations - Specific

- USG Abdomen
- Upper GI Endoscopy – to confirm growth(see ulceroproliferative growth and biopsy).
- CT plain and Contrast enhanced to demarcate
size and location of lesion,
to assess metastatic lesion,
to assess secondary nodal deposits.
- Endoscopic Ultrasound – for T N staging, USG abdomen.
- Barium meal, X ray abdomen erect.

Routine investigations**Treatment:**

CA Stomach:

If operable:

Subtotal gastrectomy with D2 lymphadenectomy

Chemotherapy

Radiation therapy

Inoperable – Palliative Chemotherapy

NECK SWELLING

A 48 year old Male Mr.Vasanth ,coming from Korukkuppet , came with chief complaints of

Swelling in left side of neck – 20 days

Patient was apparently normal before 20 days then he noticed

Swelling in the left side of the neck

Insidious in onset initially it was small in size of almost marble shape which progressed to attain the current size .

H/O **Pain over the swelling** for the past 15 days,

Dull aching type,Intermittent in nature,

Radiating to ear,No aggravating and relieving factor.

H/O difficulty in swallowing

H/O increased salivation

No H/O difficulty in chewing

No H/O difficulty in opening mouth,

No H/O oral ulcers,

H/O dental caries,

No H/O variation in in size of the swelling or pain during eating.

No H/O deviation of angle of mouth or difficulty in closing eyes

No H/O slurred speech.

No H/O hoarseness of voice

No H/O ear discharge ,deafness

No H/O nasal bleed

No H/O dyspnea

No H/O cough with hemoptysis.

No H/O altered bowel habits.

No h/o jaundice , haematemesis, malena .

No h/o Trauma

No h/o fever, evening rise of temperature,night sweats,

H/O loss of weight,H/O loss of appetite.

No H/O any other swelling

Past History / Personal History / Family and Rx History

General Examination & Vital signs

Local examination - Inspection

Swelling in lateral aspect of the upper part of the neck on the left side occupying upper 1/3 rd of sternocleidomastoid approx. 8*6 cm Oval in shape

Extent:

Upper border : 1.5 cm below mastoid process

Lower border : 2cm below angle of mandible

Anterior :6cm from angle of mouth

Posterior :5cm from midline of the back

Surface: smooth,Skin over the swelling: stretched

Margin:defined ,No scars.,No sinuses,No dilated veins,No visible pulsation

Retromandibular groove – not obliterated.

Restriction of movement on turning neck to left side

Palpation

Not Warmth,Tender

Size:8*7 cm ,Shape : oval,Surface : Nodular Hard in consistency Margin: well defined

Mobility: Restricted

Skin not pinchablePlane of the swelling: deep to deep fascia.

Carotid pulsation : Felt on both sides on normal position

Mouth

Oral hygiene - Poor,Halitosis – Present,

Lips – Normal,Labial sulcus - Normal.

Gingivobuccal sulcus : Normal,Gums: normal

Buccal mucosa : Nicotine stains present,

Dental caries : Present

Floor of the mouth : Normal

Anterior 2/3 rd of tongue : Normal, Mobility : Normal No Deviation.

Hard palate : Normal.

Anterior tonsillar pillar : Normal.

Dental formula:

Posterior pharyngeal wall : Normal.

Retromolar trigone : Free.

Opening of mouth : Normal (admits 3 fingers)

Bimanual palpation: deep lobe is not palpable.

Bidigital palpation : No stricture or stone felt.

No discharge from Stensons duct

EAR - No Discharge, No Perforation, No Polyp

Nose - No Discharge, No Polyp, No Nasal deviation

Scalp - No Ulcer, No swelling

Thyroid - Not palpable

Abdomen – Soft, No Palpable mass, Non – tender, No organomegaly

Spine & Cranium – Normal

RS – Bilateral air entry, Normal vesicular breath sounds

CVS – S1, S2 heard, no murmurs, no added sounds

CNS – No focal neurological deficit.

Per rectal examination – To be done

Examination of lymph nodes

Preauricular, postauricular, occipital nodes - not felt

Cervical node on opposite side : normal

Left Supraclavicular fossa : Free

Axillary area : Normal

Inguinal region : Normal

Diagnosis

A case of neck secondaries with unknown primary probably involving level 2,3 nodes of staging TxN3M0.

Differential Diagnosis

TUBERCULOUS LYMPHADENITIS

LYMPHOMA

Specific Investigations

USG neck

CECT neck

FNAC of Neck node.

Triple endoscopy

USG abdomen

Blind biopsy

PET scan

Routine investigations

Treatment

NECK SECONDARIES WITH UNKNOWN PRIMARY:

Radiotherapy

Radical neck dissection followed by chemotherapy